

F A M O D I N E

COMPOSITION:

Famodine Tablet 20mg: Each tablet contains Famotidine 20mg

DESCRIPTION:

Famodine Tablet 20 mg is a yellow caplet, 11 mm with scored on one side and marking 'd 861' on another side. The Tablet can be split.

PHARMACODYNAMICS:

GI effects: Famotidine is a competitive inhibitor of Histamine H₂-receptors. The primary clinically important pharmacological activity of Famotidine is inhibition of gastric secretion. Both the acid concentration and the volume of gastric secretion are suppressed by Famotidine, while changes in pepsin secretion are proportional to volume output.

PHARMACOKINETICS: Famodine is incompletely absorbed. The bioavailability of oral doses is 40-45%. Bioavailability may be slightly increased by food, or slightly decreased by antacids; however, these effects are of no clinical consequence. Famodine undergoes minimal first-pass metabolism. After oral doses, peak plasma levels occur in 1-3 hours. Plasma levels after multiple doses are similar to those after single doses. Fifteen to 20% of Famodine in plasma is protein bound. Famodine has an elimination half-life of 2.5-3.5 hours. Famodine is eliminated by renal (65-70%) and metabolic (30-35%) routes. Renal clearance is 250-450mL/min, indicating some tubular excretion. 25% to 30% of an oral dose is recovered in the urine as unchanged compound. The only metabolite identified in man is the S-oxide.

INDICATIONS:

- Duodenal ulcer
- Benign gastric ulcer
- Zollinger-Ellison Syndrome
- Prevention of relapses of duodenal ulceration
- Short term (no more than 12 weeks) symptomatic relief of gastroesophageal reflux not responsive to conservative measures
- Healing of oesophageal erosion or ulceration associated with gastroesophageal reflux disease.
- Prevention of relapses of symptoms and erosions or ulcerations associated with gastroesophageal reflux disease.

RECOMMENDED DOSAGE:

Famodine can be taken with or without food

Duodenal ulcer:

Initial therapy: The recommended dose of Famotidine is two tablet of 20mg famotidine to be daily taken at night. Treatment should be given for four to eight weeks, but the duration of treatment may be shortened if endoscopy reveals that the ulcer has healed. In most cases of duodenal ulcer, healing occurs within four weeks on this regimen. In those patients whose ulcers have not healed completely after 4 weeks, treatment should be continued for a further 4 weeks period.

Maintenance therapy: For the prevention of recurrence of duodenal ulceration, it is recommended that therapy with Famotidine be continued with a dose of one 20 mg tablet daily taken at night.

Benign Gastric Ulcer: The recommended dose of Famotidine is 2 tablet of famotidine 20mg daily, taken at night. Treatment should be given for four to eight weeks, but the duration of treatment may be shortened if endoscopy reveals that the ulcer has healed.

Zollinger-Ellison Syndrome: Patients without prior antisecretory therapy should be started on a dose of 20mg every six hours. Dosages should be adjusted to individual patient needs and should be continued for as long as indicated clinically. Doses up to 480mg daily have been used for up to one year without the development of significant adverse effects or tachyphylaxis. Patients who have been receiving another H₂ antagonist may be switched directly to Famotidine at a starting dose higher than that recommended for new cases; this starting dose will depend on the severity of the condition and the last dose of the H₂ antagonist previously used.

Dosage adjustment for patients with moderate to severe renal insufficiency:

Dosage adjustment is required for patients with moderate to severe renal insufficiency. Since CNS adverse effects have been reported in patients with moderate to severe renal insufficiency, to avoid excess accumulation of the drug, the dose of famotidine may be reduced to half the recommended dose or the dosing interval may be prolonged to 36 - 48 hours as indicated by the patient's clinical response

Gastroesophageal Reflux Disease: The recommended dosage for the symptomatic relief of gastroesophageal reflux disease is 20 mg of famotidine taken orally twice daily.

For the treatment of oesophageal erosion or ulceration associated with gastroesophageal reflux disease: The recommended dosage is 20 mg of famotidine twice daily.

Maintenance of Therapy: For the prevention of recurrence of symptoms and erosions or ulcerations associated with gastroesophageal reflux disease, the recommended dosage is 20 mg of famotidine twice daily.

CONTRAINDICATIONS:

Hypersensitivity to any component of these products. Cross sensitivity in this class of compounds has been observed. Therefore, Famodine should not be administered to patients with a history of hypersensitivity to other H₂-receptor antagonists.

WARNINGS AND PRECAUTIONS:

Gastric Neoplasm: Gastric malignancy should be excluded prior to initiation of therapy for gastric ulcer with Famotidine. Symptomatic responses of gastric ulcer to therapy with famotidine does not preclude the presence of gastric malignancy.

Paediatric Use: Safety and effectiveness of Famotidine in children have not been established.

Use in the Elderly: No dosage adjustment is required based on age alone. As elderly patients are more likely to have decreased renal function, care should be taken in dose selection in this patient group, and it may be useful to monitor renal function.

Renal Insufficiency: CNS adverse effects have been reported in patients with moderate (creatinine clearance < 50 mL/min) and severe (creatinine clearance < 10 mL/min) renal insufficiency. Consequently, the famotidine dosage should be reduced in patients with moderate or severe renal insufficiency. If endoscopy reveals that the ulcer has healed. In most cases of duodenal ulcer, healing occurs within four weeks on this regimen. In those patients whose ulcers have not healed completely after 4 weeks, treatment should be continued for a further 4 weeks period.

Effects on Ability to Drive and Use Machine: Famotidine may cause certain adverse effects such as dizziness, confusion, or hallucinations and therefore, patients should know how they react to Famotidine before they operate an automobile or machinery or engage in activities requiring mental alertness and coordination.

INTERACTIONS WITH OTHER MEDICAMENTS:

No drug interactions have been identified. Famotidine does not interact with the cytochrome P₄₅₀ – linked drug metabolizing enzyme system. Compound metabolized by this system, which have been tested in man have included warfarin, propranolol, theophylline, phenytoin, diazepam, aminopyrine and antipyrine. Indocyanine green as an index of hepatic blood flow and / or hepatic drug extraction has been tested and no significant effects have been found.

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PREGNANCY:

Use in Pregnancy: Famotidine is not recommended for use in pregnancy, and should be prescribed only if clearly needed. Before a decision is made to use Famotidine during pregnancy, the physician should weigh the potential benefits from the drug against the possible risks involved.

Use in Lactation: Famotidine is detectable in human milk. Nursing mothers should either stop this drug or stop nursing.

SIDE EFFECTS:

Famotidine has been shown to be generally well tolerated. Headache, dizziness, constipation and diarrhoea have been reported at a frequency of greater than 1% and may be causally related to Famotidine. Rarely reported events included dry mouth, nausea and/or vomiting, rash, abdominal discomfort or distension, anorexia, fatigue, pruritus, urticaria, liver enzymes abnormalities, cholestatic jaundice, anaphylaxis, angioedema, arthralgia, muscle cramps, reversible psychic disturbances including depression, anxiety disorders, agitation, confusion, and hallucinations. Toxic epidermal necrolysis has been reported very rarely with H₂ receptor antagonists. In addition to the above side effects, AV block has been reported very rarely with H₂-receptor antagonists administered intravenously.

The following side effects have been reported, however, a causal relationship to therapy with Famodine has not been established: decreased libido, paraesthesia, somnolence, insomnia, grand mal seizure, thrombocytopenia, pancytopenia, leukopenia, agranulocytosis. Rare cases of impotence and rare cases of gynecomastia have been reported.

SYMPTOMS AND TREATMENT OF OVERDOSE: There is no experience to date with overdosage. Doses of up to 800 mg daily have been used in a small number of patients with Zollinger-Ellison Syndrome for more than a year without development of significant adverse effects.

The usual measures to remove unabsorbed materials from the gastrointestinal tract clinical monitoring and supportive therapy should be employed.

PACK SIZES / PACKING:

Famodine Tablet 20 mg: blister pack of 10 x 100's.

STORAGE CONDITIONS: Store below 30°C. Protect from light. Keep out of reach of children. *Jauhkan daripada kanak-kanak.*

SHELF LIFE:

Please refer to outer package

Manufacturer and Product Registration Holder:

DUOPHARMA (M) SDN BHD (42491 M)
Lot 2599 Jalan Seruling 59 Kaw 3 Taman Klang Jaya,
41200 Klang Selangor Darul Ehsan MALAYSIA
15000